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Unit- 10:

SEXUALITY & REPRODUCTION PATTERN

Human Sexuality

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- **Human sexuality** is X-zed by *physical, social, psychological, and spiritual attributes* that are associated with one's gender.
- **Sexuality** is expressed in terms of *physical structure, physiologic functioning, attitudes and values, knowledge and behavior* that result from inherited X-tics and social learning.
- **Sexuality** includes the function of the sexual organs plus a person's perceptions of his or her own functioning and sexual expression and preference.

Human Sexuality...

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- **Sex** is a term most commonly used to denote biological male or female stratus, but it is also used to describe specific sexual behavior such as sexual intercourse.
- **Sexual health** includes one's freedom from *physical* and *psychological* impairment, the awareness of open and positive *attitudes* toward sexual functioning; accurate *knowledge* of sexuality and congruency among gender identity and role.

Five Features of Sexuality

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- ☐ Sensuality
- ☐ Intimacy
- ☐ Sexual Identity
- ☐ Reproduction
- ☐ Sexualization

Features of sexuality....

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- **SENSUALITY:** *awareness and acceptance of our own body.*
 - Includes knowledge of anatomy & physiology, understanding sexual response, body image, satisfaction of skin, attraction template, and fantasy.
- **INTIMACY:** *experiencing emotional closeness to another.*
 - This include caring, sharing, risk taking, vulnerability, and self disclosure.
- **SEXUAL IDENTITY:** *process of discovering who we are in terms of sexuality.*
 - These may be gender roles, orientation, self esteem & confidence level, r/ships with family & friends, roles as child & adult, and a perception of self as male/female

Features of sexuality....

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- **REPRODUCTION:** *values, attitudes & behaviors relating to reproduction.*
 - These are Contraception & fertility issues, lifestyles, STIs, anatomy & physiology.
- **SEXUALIZATION:** *use of sexuality to influence, control or manipulate.*
 - These are appearance & body language, advertising, style of dress, movies, talk shows & media, harassment & sexual assault, paraphilias (voyeurism, exhibitionism...)

Values

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- **VALUES:** the **qualities in life** which are deemed important or unimportant, right or wrong, desirable or undesirable
- **MORAL VALUES:** relate to our conduct with and treatment of other people, more than just right or wrong, looks at the whole picture
- **SEXUAL MORAL VALUES:** relate to the rightness and wrongness of sexual conduct and when and how sexuality should be expressed
- **SOURCES OF SEXUAL VALUES:** we acquire our sexual values from our social environment

Characteristics of sexual health

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- Sexual health includes biologic, psychological, socio-cultural, and spiritual components.
- Characteristics of sexually healthy people are:
 - ▣ Expression of a positive body image
 - ▣ Congruence b/n biologic sex, gender identity and gender role behavior
 - ▣ Behavior consistent with self-concept
 - ▣ Awareness of own sexual feeling& attributes

X-tics of sexual health...

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- ▣ Capacity for physical and psychological responsiveness, w/c is enhancing to self and others
- ▣ Comfort with a range of sexual behavior and life style
- ▣ Acceptance of responsibility for pleasure, reproduction and physical safety
- ▣ Ability to create and maintain effective interpersonal relationship with both sexes

Ctd....

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- **Four areas of competencies incorporate sexuality to standards of practice .**
- **Comfort with own sexuality,**
- **Excellent communication skills,**
- **Knowledge about sexuality,**
- **Role models to demonstrate integration of sexuality to practice .**

SEXUAL ORIENTATION & ATYPICAL BEHAVIOR

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A. SEXUAL ORIENTATION: direction of one's sexual interests.

- ❖ This can be toward members of the same gender, opposite gender or both genders.

These are:

- ❑ **HETEROSEXUAL:** attraction & preference for romantic relationships with other gender
- ❑ **HOMOSEXUAL:** attraction & preference for romantic relationships with same gender
- ❑ **BISEXUAL:** attraction & interest for romantic r/ships with both genders

B. ATYPICAL BEHAVIOR

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- ❑ Sexual behaviors which are unusual or abnormal
- ❑ Sexual arousal involving a preference for non-genital sexual outlets
- ❑ Patterns of sexual behavior or arousal that appear problematic in the eyes of the individual or society are called *paraphilias*.

Atypical bhr...

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❖ PARAPHILIAS

- ❑ Involve sexual arousal in response to unusual stimuli, **such as children, non consenting persons, nonhuman objects, or pain & humiliation**
- ❑ Diagnosed when **sexual fantasies, urges or behaviors cause significant distress or interfere with a person's ability to function in everyday tasks**

PARAPHILIAS

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include:

- ☐ Fetishism
- ☐ Partialism
- ☐ Transvestism
- ☐ Exhibitionism
- ☐ Voyeurism
- ☐ Frotteurism
- ☐ Sadism
- ☐ Masochism
- ☐ Zoophilia
- ☐ Necrophilia
- ☐ Nymphomania
- ☐ Satyriasis

Types of paraphilias...

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- **FETISHISM:** when sexual arousal is caused by an **inanimate object**.
E.g. rubber, leather, silk, high-heeled shoes, panties...
- **PARTIALISM:** related to fetishism; exaggerated sexual arousal **to a particular body part**.
E.g. Feet, buttocks, breasts...
- **TRANSVESTISM:** when a person repeatedly **cross-dresses** for sexual arousal or is bothered by recurring urges to cross-dress
E.g. most keep it a secret, ranges from one garment to many, almost always male, most are married...

Types of paraphilias...

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- **EXHIBITIONISM:** persistent, powerful urges & sexual fantasies involving **exposing one's genitals to unsuspecting strangers** for sexual arousal of oneself
 - **E.g.** “flashing”, almost always male, usually begins between ages 13-16, most are not a physical threat...
- **VOYEURISM:** strong, repetitive urges to **watch unsuspecting strangers who are naked, undressing or having sex.**
 - **E.g.** almost always male, masturbate during or after, usually begins before age 15, not usually violent, many lack social & sexual skills...

Types of paraphilias...

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- **FROTTEURISM:** recurring, powerful urges to rub against or touch a non-consenting person.
 - E.g. “mashing”; almost always male, many women do not realize it has occurred
- **SADISM:** the desire or need to inflict pain or humiliation on others for sexual arousal
- **MASOCHISM:** the desire or need for pain or humiliation to be inflicted on oneself for sexual arousal

Types of paraphilias...

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- When sadism and masochism practiced together the act is called **sadomasochism**.
- It may involve being tied up, biting, hitting, spanking (brisk or fast), whipping, pinching, and other activities.
- **ZOOPHILIA**: repeated sexual urges & fantasies involving sexual contact with animals
 - Men → farm animals
 - Women → household pets

Types of paraphilias...

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- **NECROPHILIA:** the desire for sexual activity with corpses (cadaver)
- **NYMPHOMANIA:** an excessive sex drive or sexual appetite in women that is insatiable
- **SATYRIASIS:** an excessive sex drive or sexual appetite in men that is insatiable
- **PEDOPHILIA** is a term used to describe the practice of adults gaining sexual fulfillment by sexual acts with children.

Types of paraphilias...

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- **SEXUAL HARASSMENT:** “any unwelcomed verbal or physical advance or sexually explicit statement – such as *leers*, pats, grabs, jokes, requests for dates, and even rape that interferes with your ability to do your job by making you feel humiliated, intimidated or uncomfortable”.
- It is important for nurse to recognize that harassment can often be effectively stopped when confronted early.

Sexual Expression

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- The methods by which people gain satisfaction through sexual stimulation are varied.
- Touch, smell, sight, sounds, feelings, thoughts, and fantasy (an idea with no basis in reality) can all contribute to sexual fulfillment in any form of expression chosen by individuals.
- Feelings of love for another person are closely associated with desire.

Sexual Stimulations

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- Forms of sexual stimulation include: kissing, hugging, striking, squeezing, breast stimulation, manual stimulation of the genitals, oral-genital stimulation, and anal stimulation.
- Sexual stimulation may be physical or psychological.
- Erotic stimulation through the use of films, magazines, and photographs is also common.

Masturbation

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- Masturbation is a technique of sexual expression in which an individual practices self-stimulation.
- Many myths (false belief) and misinformation surround the issue of masturbation.
- Masturbation is a means of learning what is preferred by a person during stimulation and what feels good.

Masturbation...

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- People masturbate regardless of sex, age, or marital status.
- People may not masturbate b/c of sense of guilt or wrongness associated with self-stimulation.
- Masturbation is not “dirty” and will not lead to blindness nor will cause insanity (extremely foolish or irrational).

Factor affecting sexuality

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- Culture
- Religion
- Ethics
- Life style
- Health status

Sexual Intercourse

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- Heterosexual genital intercourse is the most common image that comes to mind when sexuality is mentioned.
- The act of intercourse (coitus or copulation) usually begins by stimulation of the senses in some way, followed by a period of activity known as **foreplay**.
- **Petting** is part of foreplay and can be simple stroking of the breasts, arms back and rack without genital involvement or may lead to mutual masturbation and orgasms.

SEXUAL RESPONSE

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- The **sexual response** is a **complex interplay of psychological and physiologic phenomena** influenced by a number of variables, including daily stress, illness, and crises
- The changes that occur during sexual excitement are similar for men and women

Sexual response...

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- **Masters and Johnson** described the sexual response in to 5 phases as:
 - i. Pre-excitement,*
 - ii. Excitement,*
 - iii. Plateau,*
 - iv. Orgasmic, and*
 - v. Resolution phase*

Phase 1: Pre-excitement phase

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- At this phase, the individual may fantasize about or wish to actually engage in sexual activities mediated by the **level of circulating androgens** and their effect on cells in the central and peripheral nervous systems
- **Psychic stimuli** initiating a neurologic response **via the cortical pathway and mediated by sympathetic and parasympathetic nerves**
- **Tactile stimulation** initiating a neurologic response **via a reflex through the sacral spinal cord.**
- This response includes **thickening and tensing of the scrotal skin, elevation of the scrotal sac, shortening of the spermatic cord, and increase in size and elevation of testes**

Phase 2: Excitement phase

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- ❑ In response to stimulation ***Vasocongestion in the genital area*** and; the ***clitoris becomes congested and vaginal lubrication increases from secretions of the cervix, Bartholin's glands, and vaginal walls.***
- ❑ In some women, vaginal lubrication can occur within seconds during this phase, whereas other women take much longer to achieve lubrication
- ❑ The external genitalia swell and clitoris increase in size, becomes erect, and is increasingly more sensitive

- ❑ In men, ***penile erection occurs secondary to the rapid influx and retention of blood in the spongy tissues of the penis***
- ❑ Erection results from the filling of the large venous sinuses within the erectile tissue of the penis.
- ❑ The loose skin of the penis becomes tight as a result of the intense venous congestion
- ❑ The penis and the urethra are essential to the transport of sperm in the vagina and the cervix during intercourse

Phase 3: Plateau phase

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- ❑ Is the natural extension of the excitement phase the ***erection is maintained, and a small increase in diameter*** occurs as a result of a slight increase in congestion and increase in testicle size.
- ❑ In women the ***vagina expands and the uterus is elevated, labia minora take on a deeper hue and the tissues around the nipples may swell***
- ❑ The clitoris retracts under the clitoral hood.
- ❑ The ***opening to the vaginal decreases in size*** related to the engorgement of surrounding tissues

Phase 4: Orgasmic phase

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- ❑ During this phase sexual pleasure peaks
 - In men, orgasm is usually accompanied by ejaculation or ejection of seminal fluid in response to the contraction
 - The subsequent contraction of the penile and urethral musculature during the orgasmic phase propels the sperm outward through the meatus.
 - This process is termed as **ejaculation**; sperm are released into the ductus deferens during contraction.

- ❑ Orgasm is X-zed by the rapid release of the vasocogestion and muscular tension (myotonia)
- ❑ Sperm advances through the urethra, where fluids from the prostate and seminal vesicles added to the ejaculate
- ❑ The sperm continue their path through the urethra, receiving a small amount of fluid from the Cowper's glands, and are finally ejaculated through the urinary meatus
- ❑ Contractions of the pelvic floor occur in conjunction with the closure of the internal sphincters of the bladder, which prevents retrograde ejaculation or discharge of the seminal fluid into the bladder rather than outside through the urethra

- ❑ The rapid release of muscular tension (through rhythmic contraction) occurs primarily in the penis, prostate gland, and the seminal vesicles
- ❑ In women during the orgasmic phase, contractions occur in the uterus from the fundus to the lower uterine segment
- ❑ There is a slight relaxation of the cervical os, which helps the entrance of the sperm, and rhythmic contraction of the vagina
- ❑ Muscular tension is rapidly released through rhythmic contractions in the clitoris, the vagina, and the uterus

- ❑ The cervical os may contract and open slightly with an orgasm
- ❑ An all over warmth and a throbbing sensation in the pelvis is experienced
- ❑ Some women ejaculate a fluid that is similar to the male's prostatic fluid from the Bartholin's gland at the time of orgasm
- ❑ Muscles in the pelvic floor and rectal sphincter contract, forcing pooled blood out of engorged blood vessels and relieving pelvic tension

- In both men and women the HR, BP, and RR increases.
- Myotonia, or muscular tension, develops throughout the body, especially in the thighs, abdomen, and pelvic area
- Breast enlarges and nipples may become erect because of involuntary muscle fibers contraction in the areola. A flush may appear on the face
- ❖ *It is important to note that every sexual encounter will terminate in an orgasm for both partners*

Phase 4:Resolution phase

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- It is the stage in w/c the orgasm is gradually disappears/resolved

Potential cause for altered sexuality

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1. Pregnancy, infertility, and abortion:

- ☐ Pregnancy clearly alters sexual functioning.
- ☐ Infertility is often a stressor on couple's sexual relationship.
- ☐ Females may fear of another unwanted pregnancy

2. Environment: hospitalization, or crowded environment.

Potential cause for altered sexuality....

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- 3. Illness:** cardiac problem, STI, Spinal injury, etc.
- 4. Medications:** Antihypertensive, psychotic tranquilizers, antidepressants and alcohol
- 5. Surgery:** cesarean birth, hysterectomy, mastectomy.

Manifestations of altered sexuality

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- **Sexual abuse:** being sexually abusive to others such as children, spouse or unknown person.
- **Inhibited sexual desire:** when the person's partner is not satisfied with the frequency of sexual relationships.
- **Impotence:** inability to attain or maintain an erection long enough to have satisfactory intercourse.

Manifestations

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- **Dyspareunia:** is painful intercourse caused by organic problems, including lack of adequate lubrication at the opening to vagina or within the vaginal walls; drugs like antihistamines, tranquilizers or estrogen deficiency.

Manifestations.....

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- **Vaginismus:** is resulted from the involuntary contraction of the muscles surrounding the vaginal orifice such that penetration may be impossible and very painful.
- It is the result of psychological problems, namely fear of penetration due to a negative association with it, such as rape and sexual abuse or fear of men or of sexual intercourse.

Application of the Nursing Process

Assessment: Subjective assessment

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- *Functional pattern identification:*
 - ▣ Assess the general sexual relationship
- *Risk factor identification:*
 - ▣ Pregnancy, Infertility,
 - ▣ Abortion,
 - ▣ Environment,
 - ▣ Illness, Surgery etc.

- Objective Ass't
- Nursing Dx
- Planning
- Implementation
- Evaluation

UNIT- 11

VALUES AND BELIEFS

PATTERN

SPIRITUAL HEALTH

Normal spiritual Function

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- The spiritual dimension is a quality that goes beyond religious affiliation that strives for inspiration, reverence (deep respect), awe (fearfulness), meaning and purpose, even for those who do not believe in any god.
- The spiritual dimension tries to be in harmony with the universe, strives for answers about the infinite, and comes in to focus when the person faces emotional stress, physical illness, or death.

Normal spiritual Function

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- Although all people have this dimension within their being, not all have the same depth or intensity of it in their lives.
- The spirit or spirituality refers to the quality or essence that pervades, integrates, and transcends one's biopsychosocial nature.

Characteristics of Normal Spirituality

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- The major characteristics of normal spirituality include a sense of wholeness and harmony within one's self, with others, nature and with god, or one's higher power as one defines it.

X-tics of Normal Spirituality

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Holism

- Is the position of reviewing the universe as a system of harmonious interconnectedness rather than a sum of isolated parts
- Not only integrates mind-body, but also emphasizes spirit.
- A holistic approach is grounded in the recognition that the spiritual struggle is a valid and important aspect of health and health care.

X-tics of Normal Spirituality

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Spiritual Need

- Represents a normal expression of a person's inner being that seeks meaning in all experience and a dynamic relationship with self, others, and to the supreme other as defined by the person.
- Are derived through affective experiences of faith, hope, love, and positive experiences that serve as catalysts of synthesis and meaning.
- A composite list of spiritual needs includes trust, forgiveness, love and relatedness, faith, creativity and hope, meaning and purpose, and grace.

X-tics of Normal Spirituality

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Spiritual Quest

- Life may be looked on as a spiritual quest, not only to answer the philosophic questions, but also to seek a higher level of consciousness or a deeper awareness of spiritual life.
- Spiritual health includes “ . . . our ability to discover and articulate our own basic purpose in life, learn how to experience love, joy, peace and fulfillment. . . .”

X-tics of Normal Spirituality

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□ **Spiritual Well- being**

- Is a condition marked by an affirmation of life, peace, harmony, and a sense of interconnectedness with God/Allah, self, community and environment that nurtures and celebrates wholeness.

Factors Affecting Normal Spiritual Expression

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- **Culture:** Attitudes, beliefs, and values arise out of one's socio-cultural back ground.
- Usually, but not always, people follow the spiritual and religious traditions of the family.
- Often, a person's religious preference is tied to the families' ethnic background.
- Whatever the religious tradition or belief system the person follows, the inner spiritual experience is uniquely personal.

Factors...

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- **Gender:** One's spiritual expression also depends on the societies and religious group's beliefs and teachings about gender or expected behaviors in male and female roles.

Factors....

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- **Previous experience:** During a crisis period, past coping styles or learned ways of handling situations are likely to be in evidence.
- These coping patterns can be healthy and adaptive, or they can be maladaptive.
- Additionally, the life experiences in general are an influence. Such experiences may be related to age, but not necessarily.

Factors....

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- **Crisis:** A crisis may strengthen a person's spirituality.
- This often happens when people face death, for example, as a soldier on a battlefield or a terminally ill patient

Altered Spiritual Function: Potentials cause for Altered Spiritual Function

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- **Crisis and Change:** Just as crisis may strengthen one's faith, it may also deal it a blow.
- Many people do not consciously reflect on their personal philosophy of life.
- They live life as if it were going to continue forever. "Often it is not until the crisis, illness, aging, loss, limitation, or suffering occurs that the illusion [or security] is shattered
- Therefore, illness, suffering, aging, loss and ultimately death, by their very nature, become spiritual experiences as well as physical and emotional experiences.

Altered Spiritual Function: Potentials cause for Altered Spiritual Function

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- **Separation from Spiritual Ties:** The experience of being a patient in the hospital or a resident in a retirement or nursing home can be shattering initially.
- One is isolated, to some extent, from personal freedom, personal privileges, and social support systems.
- This separation from spiritual ties places the person at risk for altered spiritual function.

Altered Spiritual Function: Potentials cause for Altered Spiritual Function

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- **Moral Issues Regarding Therapy:** The choice to treat may be difficult to make if the religious beliefs say “no” and the health care system says “yes”.
- E.g. abortion, blood transfusion, taking birth control.

Altered Spiritual Function: Potentials cause for Altered Spiritual Function

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- **Inadequate or Inappropriate care:** Nurse's insecurity in their own spiritual lives, assigning less value and importance to spiritual care, having little or no educational preparation in spiritual care, or believing that it is the clergy's territory can place the patient at increased risk of altered spiritual function.

Manifestations of altered spiritual Function

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Verbalization of Distress

- The person suffering spiritual dysfunction may verbalize that distress or express a need for help. The manifestation may be precise.
- “I feel guilty because I should have realized earlier he was having a heart attack.”
- A patient may ask the nurse to pray for him or her or to notify a spiritual leader of his or her illness.
- A person may state that he/she misses Sunday church service and the beautiful music of the choir, or may say “I have never missed a service in 20 years”

Manifestations of altered spiritual Function

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Altered Behavior

- A patient who is nervous about the outcome of diagnostic test or who shows anger after hearing the results may be suffering from spiritual distress.
- Sleeplessness or lack of concentration, guilt, fear, depression, and anxiety may indicate altered spiritual function.

Application of Nursing Process

Assessment: Subjective Data

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- *Functional Pattern Identification:* Is religion or God significant to you? To whom do you turn when you need help?
- *Dysfunction Identification:* Depends on the nurse's observation of the client's verbal and nonverbal responses to the nursing history interview

Application of Nursing Process

Assessment: Objective Data

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- General appearance, facial expression, anxiety, crying; materials such as religious articles, books, cards, and pictures also indicate the spiritual dimension, as do visitors from the church or clergy.

Application of Nursing Process

Nursing Diagnosis

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- *Spiritual distress (distress of the human spirit)*

Application of Nursing Process

Nursing Interventions

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- Listen empathetically and sensitively
- Encourage verbalization about faith and hope
- Identify the patient's personal coping mechanisms
- Refer to appropriate resources for spiritual support

THANK YOU SO MUCH!!

Let's Read to Make a D/ce



END!

**“ Integrity is better capital than money.
You can accumulate it just like money,
and you can use it just like money,
but it goes further, and is enduring. ”**

Strive Washington

**Use any language you want
Please Write A Comment
regarding learning & teaching
process**

- 1. My Weakness, any weakness**
- 2. My Strength, any strength**
- 3. Methods of teaching: how was and how it should be**